

Pale Skin - Female (Adult)

Algorithm 1112 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT-only assessment of pallor that first excludes shock and active bleeding, then separates adult possible anaemia from pediatric pallor requiring age-appropriate investigation.
- Adult patient.
- Female.

Initial assessment

1. [DURATION] Did the pallor appear suddenly or gradually, and how different is it from the person's usual skin, lips, palms, nail beds, or inside the lower eyelid?
2. [YES_NO] Is there active heavy bleeding, vomiting or coughing blood, black or bloody stool, bleeding in pregnancy or after birth, recent trauma or surgery, or possible internal bleeding?
3. [YES_NO] Is the person cold or clammy, faint or confused, very weak, difficult to wake, breathing rapidly or with difficulty, or showing a rapid weak pulse or collapse?
4. [OPEN_TEXT] Is there tiredness, breathlessness, palpitations, dizziness, headache, fever, bruising, repeated infections, poor appetite, poor growth, jaundice, heavy periods, pregnancy, or known anaemia or blood disorder?
5. [YES_NO] For a baby or child, is there reduced feeding, reduced activity, abnormal sleepiness, breathing difficulty, poor growth, unexplained bruising or bleeding, persistent fever, or caregiver concern that the child is significantly unwell?

Triage and redirect questions

1. Disposition level 100

Is there active heavy bleeding or suspected internal bleeding, or pallor with collapse, confusion, reduced responsiveness, cold clammy skin, severe weakness, rapid or difficult breathing, or a rapid weak pulse suggesting shock?

NHS first-aid guidance identifies pale cold clammy skin, rapid shallow breathing, weakness and dizziness as shock features requiring immediate emergency help. Active bleeding or suspected internal bleeding must be managed directly and must not redirect to the missing Bleeding protocol.

1. Disposition level 78

With no shock or active bleeding, is this a baby, child, or young person with persistent unexplained pallor, or is any person significantly breathless, tachycardic, faint, rapidly worsening, pregnant or recently postpartum, or known to have a haemoglobin disorder?

Pallor in children has a broader differential than dietary iron deficiency and requires age-appropriate assessment; NICE advises very urgent full blood count for unexplained pallor in children and young people. Significant symptoms, pregnancy/postpartum status, or a known haemoglobin disorder also require prompt clinical review.

1. Disposition level 50

For an adult with all emergency and urgent features excluded, is there gradual pallor with tiredness, mild breathlessness, headache, or palpitations suggesting possible anaemia?

NHS.UK lists pallor, tiredness, breathlessness, headache and palpitations as possible iron-deficiency anaemia symptoms, but diagnosis requires blood testing and evaluation of the cause rather than assumed iron treatment.

Advice

Qatar shock or bleeding emergency response

- Call Qatar emergency services on 999 and use emergency ambulance transport; do not allow self-driving. If there is visible external bleeding, apply firm direct pressure with a clean dressing unless an object is embedded; do not remove an embedded object. Keep the person warm and still. If injuries and breathing allow, lie them down; do not give food or drink. Do not use the missing Bleeding protocol as a redirect. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: continued or hidden bleeding; worsening weakness; collapse or loss of consciousness; confusion; rapid or difficult breathing.

Urgent age-appropriate pallor assessment

- Arrange prompt in-person assessment and blood testing through an approved Qatar adult, maternity, or pediatric pathway. Unexplained pallor in a child must not be treated as presumed dietary iron deficiency without assessment. Do not start iron solely from this protocol; iron can be harmful when unnecessary or overdosed. Call 999 if bleeding, collapse, reduced responsiveness, or breathing difficulty develops.
- Call Back If: active bleeding; fainting or collapse; increasing breathlessness; reduced responsiveness; rapid deterioration.

Adult possible-anaemia follow-up

- Arrange a PHCC assessment and blood test for possible anaemia and its cause. Do not assume iron deficiency or start iron solely because skin appears pale. Seek urgent reassessment if breathlessness, palpitations, dizziness, bleeding, or weakness increases.
- Call Back If: symptoms worsen; shortness of breath increases; bleeding develops; fainting.

First aid

- Call Qatar emergency services on 999 and use emergency ambulance transport; do not allow self-driving. If there is visible external bleeding, apply firm direct pressure with a clean dressing unless an object is embedded; do not remove an embedded object. Keep the person warm and still. If injuries and breathing allow, lie them down; do not give food or drink. Do not use the missing Bleeding protocol as a redirect. Call Qatar 999 now,

follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (accessed 2026-07-25)
- NHS.UK, "First aid: shock and severe bleeding", <https://www.nhs.uk/tests-and-treatments/first-aid/> (accessed 2026-07-25)
- NICE, "Suspected cancer: recognition and referral", NG12, <https://www.nice.org.uk/guidance/ng12> (pallor in children and young people)
- Great Ormond Street Hospital, "Anaemia", <https://www.gosh.nhs.uk/conditions-and-treatments/general-medical-conditions/anaemia/>
- NHS.UK, "Iron deficiency anaemia", <https://www.nhs.uk/conditions/iron-deficiency-anaemia/> (page last reviewed 26 January 2024)

Provenance

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